CANDESARTAN

Heart failure (CHARM)
Teratogenic

CHEMICAL NAME 2-ethoxy-1-({4-[2-(2H-tetrazol-5-yl)phenyl]phenyl}methyl)-1H-benzimidazole-7-carboxylic acid (as cilexetil ester)
CLASS Angiotensin II receptor blocker (ARB, prodrug)
TRADE NAMES Atacand · Blopress · Candesar

THERAPEUTIC USES / INDICATIONS

- Hypertension.
- Heart failure with reduced EF (CHARM trials) — alone or added to ACE inhibitor.
- Migraine prophylaxis (off-label).

PHARMACODYNAMICS

- Tight-binding **AT1 receptor** antagonist → long-lasting BP reduction.
- Reduces hospitalisation and mortality in heart failure.

ADMINISTRATION

- **PO** once daily with or without food.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Dizziness, headache.
- Back pain.
- Hypotension.

ADVERSE EFFECTS (SERIOUS)

- Hyperkalaemia (↑ with ACEi + MRA).
- Renal impairment.
- Angioedema (rare).

DOSAGES

- **Hypertension:** 8 mg once daily; usual 8–16 mg; max 32 mg.
- **Heart failure:** 4 mg once daily, double every 2 weeks to 32 mg once daily.
- Hepatic/renal impairment: start 4 mg.
- **Children 6–17 yr:** 4–8 mg daily by weight.

CONTRAINDICATIONS & CAUTIONS

- Pregnancy.
- Severe hepatic impairment/cholestasis.
- Aliskiren in diabetes.
- Triple RAAS blockade (ACEi + ARB + MRA); avoid.

KEY INTERACTIONS

- **K^+ supplements, MRAs:** hyperkalaemia.
- **NSAIDs:** AKI.
- **Lithium:** toxicity.

PHARMACOKINETICS

- **Onset:** 2 h; full effect 4 weeks. **Duration:** >24 h.
- Prodrug (cilexetil) hydrolysed in gut wall to candesartan.
- Excreted in urine and bile. **t½:** 9 h.

NURSING MANAGEMENT

- HF: daily weights, BP, U&E during titration.
- Hold dose if SBP <90 or K^+ >5.5.
- Monitor renal function closely if combined with ACEi.
- Postural BP.

PATIENT EDUCATION

- Take daily; dose will be increased gradually.
- Weigh yourself daily (heart failure).
- Report dizziness, swelling of face.
- Avoid pregnancy.

KEY POINTS

- Long-acting ARB; heart failure benefit.
- Avoid ACEi + ARB + MRA triple combo.
- Titrate to 32 mg in HF.
- Contraindicated in pregnancy.